



SIMON LENNANE

Tackling loneliness to improve community health

Use of healthcare by patients varies significantly. While around three in ten of our patients do not attend in any one year, nearly half of attendances to GP practices come from 10% of patients. The reasons for seeking help when others do not are complex, but one significant contributor to both healthcare access and poor outcomes is loneliness. As clinicians, knowing what resources and groups are available locally lets us connect patients to sources of support.

There are many examples of health creation around us when we look. Men's sheds provide purpose and companionship for retired men. Parks give the opportunity for activity and time together, and Parkrun is a great way to encourage exercise. Community gardens connect people to local food production in natural environments. These groups all bring enormous health benefits by improving social capital. Being able to signpost patients to local community assets is a vital skill for effective practice.

The recognition that some people will need more input has brought social prescribers into the primary care team. Social prescribing is the use of non-medical sources of support, and is an effective way to start to address the wider determinants affecting the health of our patients. Social prescribing link workers are additional roles staff offering increased support to patients as part of wellbeing teams.

Social prescribing relies on there being activity within the

local community to 'prescribe', however. It also risks medicalising support, especially where access to social prescribers has been restricted to clinicians. If the purpose is to reduce clinical demand, needing a clinician to make the referral does the exact opposite.

The benefits of a healthy society should be equally available to anyone who works with people, across health and social care and the voluntary and community sector (VCS), but signposting needs to be a two-way process. Volunteers do not have the same ability to manage risk as healthcare. Clinicians support the local VCS best by being available to review any concerns. Good communication is vital, and building links with our local wellbeing teams lets us connect our patients better to community sources of health.

The town in which I work, Ross-on-Wye, has focused on improving the health of patients by developing the local community. This is effective medicine; the town of Frome in Somerset reduced hospital admissions by enhancing community capacity. It is easier to avoid admission for our patients if we know they have neighbours able to support them through an episode of illness.

Building connections provides a safety net for us and the people around us.

Dr Simon Lennane is a GP in Ross-on-Wye and inequality lead for the local PCN. Simon's book, 'Creating Community Health: Interventions for Sustainable Healthcare' is published by Routledge. Readers can get a 20% discount using the code AFL02: www.routledge.com/9781032140971



CRYSTAL OLDMAN

District nurses have a huge role to play in wound care

On 26th April, I had the great honour of attending a launch of the Leg Club Manifesto in the House of Commons Pavilion and Terrace. The Leg Club® movement is a global initiative, designed to provide the most appropriate care for people suffering from or at risk of chronic leg disease within a social model of care.

The network of Leg Clubs strives to provide personalised and evidence based wound care. They work with local communities and nurses and they follow the core principles of the social model of care, whilst being appropriately adapted to the needs of the local community.

Ellie Lindsay is a QNI Fellow who founded the movement of Leg Clubs and is a Trustee of the Lindsay Leg Club® Foundation. Established almost 20 years ago, the Foundation facilitates and manages the growth of the network of the Leg Clubs.

In 2022, the NHS provided care to around 2.2 million patients with a wound. This is equivalent to 4.5% of the adult population, at a cost of approximately £5.3 billion.

Thinking about how this situation has arisen, the speakers suggested a range of possible reasons. For example, leg ulcers are considered a disease of older people and not one that requires much resource to manage. This was dispelled with the data on the age range of people with leg ulcers – including those in their twenties and thirties. Reference to those people with leg ulcers being potentially economically inactive was an excellent consideration; if their

conditions were not addressed, they could be out of the workforce for many years quite unnecessarily.

The wider social, mental and physical impacts of having a chronic leg wound were also addressed – with stories of people housebound for years while their ulcers were not treated appropriately, because they had not alerted their GP and had attempted to self-manage the wound.

Another astonishing fact was the lack of preventative care that is available where people are potentially at risk of leg ulcers. The event opened with a devastating story of a fit, older lady who developed itchy red legs during the pandemic, but was never assessed by a clinician with expertise in tissue viability. Within months she had developed leg ulcers which went untreated except for the pain, and she died in hospital from sepsis. This was a potentially preventable situation; the story came from the lady's daughter who wanted her story to be told so that the experience of her mother might be prevented for others.

District nurses are experts in wound care and much of the caseload of a district nursing service will be related to applying clinical expertise in tissue viability. However, with the recent release of data about the numbers of district nurses in the NHS in England dipping below 4000 for the first time in 10 years, there is a widening gap between supply and demand, leaving people with less access to the nursing care they need and creating more of a postcode lottery than ever before.

Crystal Oldman, chief executive, QNI